

CORY SHEPHERD

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Supporting statement – two applications for sick leave without pay

Recoding of two periods presently recorded as leave without pay, at the request of Metro South Payroll · 11 September 2026

1. What is applied for

Two applications for **sick leave without pay**, for the two periods identified by Metro South Payroll on 7 and 8 September 2026. The dates are Payroll's own and are not varied.

Application	Period	Days (inclusive)
1	21 June 2024 – 20 September 2024	92
2	13 December 2024 – 23 February 2025	73

The periods are presently recorded as leave without pay. Nothing else is sought by these applications, and no other period is affected.

2. Payroll has already identified the correction and the step required

On 7 September 2026 at 1:28 pm Metro South Payroll wrote to the line manager:

“Could I please confirm with you whether the following periods were intended to be recorded as Sick No Pay rather than Leave Without Pay? 21.06.2024 - 20.09.2024 / 13.12.2024 - 23.02.2025. This will affect the employee's LSL date... we would require a leave form to be submitted for both periods. Could you please confirm?”

On 8 September 2026 at 2:24 pm Ms Grace Strachan, Senior Payroll Officer, wrote to me:

*“We believe that the periods 21/06/2024 - 20/09/2024 and 13/12/2024 - 23/02/2025 **should have been booked as Sick No Pay (SNP) rather than Leave Without Pay (LWOP)**... If these periods need to be changed to Sick No Pay, new leave forms will be required for both periods. Once the leave forms are received and the leave is updated accordingly, **we can recognise the first three months of each period towards your LSL date.**”*

These applications are made so that the leave forms Payroll requires are on foot. I am not asking anyone to accept a new position; the position is Payroll's own.

3. Application 1 – 21 June 2024 to 20 September 2024

The absence was medical from the outset and is documented throughout:

- My leave record shows **sick leave on 7, 8 and 9 June 2024**, including sick leave without pay on 9 June, and leave without pay from 10 June 2024.
- A **medical certificate was issued on 28 June 2024**.
- The **work capacity certificate of Dr Peter Hawes dated 1 July 2024** records the date of injury as **18 June 2024** and that I was first seen for that injury on 1 July 2024.
- That certificate certifies **no functional capacity for any type of work over the period 1 July to 6 October 2024** – a period which contains the whole of this application from 1 July onward.
- Further work capacity certificates were issued on **7 August 2024 (Dr Ki Pang)** and on **11 August and 8 September 2024 (Dr Hawes)**.

- Review Decision 69983 records that the certificate of 1 July 2024 indicated “there was no pre-existing factor or condition”, and that this “was maintained in all later work capacity certificates”.
- My application for workers’ compensation was **lodged on 1 July 2024** and rejected on 13 September 2024. That rejection is under appeal and is not a matter for this application.

4. Application 2 – 13 December 2024 to 23 February 2025

The absence was a continuation of the same medical condition:

- **On 24 October 2024** my treating psychiatrist, Dr Ravikumar Bangalore Krishnaiah, diagnosed **Major Depressive Disorder with anxiety state** at the first consultation, and confirmed that diagnosis in writing the same day.
- **On 13 February 2025** – within this period – Dr Krishnaiah provided a report recording the continuing diagnosis, its severity and functional effect, and the treatment then in place.
- The period ends on **23 February 2025**, being the day before a return to work was confirmed for **7:00 am on 24 February 2025**.

If a work capacity certificate specific to this period is required in addition to what is already held, please let me know and I will obtain it from the treating practice. I would rather be told what is missing than have the application held.

4A. The medical evidence, and where it is held

Four work capacity certificates are attached to this statement: **Dr Peter Hawes, 1 July 2024; Dr Ki Pang, 7 August 2024; and Dr Peter Hawes, 11 August and 8 September 2024**. Together they certify **no capacity for any work over the period 1 July to 6 October 2024**, each recording the stated date of injury as 18 June 2024 and first presentation on 1 July 2024.

The same certificates are held by the Workers’ Compensation Regulator at items 7 and 8 of its list of documents, and certificates for these absences were provided to Metro South Health at the time. If anything further is required, please tell me what it is and I will provide it.

The question of how these periods came to be recorded as leave without pay rather than sick leave without pay is the question I raised with Payroll on 7 September 2026, and it remains open. **Nothing in these two applications depends on that question being answered**, and I do not ask that it be answered before the recoding is effected.

5. The effect, on Payroll’s own figures

Payroll’s calculation of 4 September 2026 records my commencement with Queensland Health as **25 March 2019**, and that eligibility would ordinarily arise on **25 March 2026**. Payroll has identified **189 days presently excluded**, being 24 days (18 May – 10 June 2022), 92 days (21 June – 20 September 2024) and 73 days (13 December 2024 – 23 February 2025), which moves the date to **29 September 2026**.

On Payroll’s stated approach – that on recoding “the first three months of each period” are recognised – each of these two periods falls **entirely** within its own first three months: three months from 21 June 2024 ends 21 September 2024, and the period closes on 20 September; three months from 13 December 2024 ends 13 March 2025, and the period closes on 23 February. On that arithmetic **165 of the 189 days return**, leaving only the 24 days of 2022, and the resulting date falls in **April 2026**. The calculation is of course Payroll’s to make.

6. Reservation

I have received no wages for work since 13 July 2026. Since 3 July 2026 I have been certified fit for duties with restrictions on the Employee Capability Checklist completed by Metro South Health, I have remained available, and I have not been provided with work. What I have been paid since 13 July has been drawn from my own leave accruals and not paid as wages.

Any leave applied for, taken or paid in that period is applied for and taken under protest and without prejudice to my claim for wages for the period I have not been provided work, and I reserve the right to seek reinstatement of the leave taken. These two applications relate to 2024 and 2025 and are made solely to give effect to the recoding Payroll has identified; they are not an acceptance of any position about 2026.

Signed



Cory Lea Shepherd
Administration Officer, Switchboard Services, Logan Hospital
Employee no. 388372

Date: 11 September 2026

Attached: work capacity certificates of Dr Peter Hawes dated 1 July 2024, Dr Ki Pang dated 7 August 2024, and Dr Peter Hawes dated 11 August and 8 September 2024.

Work capacity certificate – workers' compensation

Form 132M — Version 1

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity to work.

Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name Cory Shepherd Date of Birth 11/01/1991
Mobile number 0422438627 Claim number S23LW142013 ☒ New claim ☐ Claim is report only
Occupation (if known) Admin worker Patient's employer Qld Health

Part B - Injury details

Date of examination 01/07/2024 Patient's stated date of injury 18/06/2024 Patient was first seen at this practice/hospital for this injury/disease on 01/07/2024

The patient is/was suffering from (List all work-related diagnoses. anxiety, stress
If symptoms only, tick "Provisional diagnosis") ☐ Provisional diagnosis

Patient's stated mechanism of injury harrasment at work

Is this consistent with your clinical findings?

☒ Yes ☐ Unclear

Describe mechanism in detail ongoing breaking of workplace rules by bosses, victimizing him

Pre-existing factors or condition

aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from 01/07/2024 to 04/08/2024 to be reviewed again on 04/08/2024 ☐ No further review

Treatment needs time way from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Name/discipline Details (specify)
Specialist/GP

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F. ☐ If suitable duties available, can return to some form of work from ☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

stressed and anxious,

Estimated time to return to some form of work duties unknown

Estimated time to return to full duties unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	Note any restrictions (if relevant)	What patient can do (if "yes" is selected)
Lower limb	No		
Upper limb	No		
Hand function	No		
Spinal function	No		
Cognition/psychosocial functioning	No		
Driving a car	No		
Operating machinery/heavy vehicle	No		
Manual tasks	No		
Other	No		

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

☐ I require a suitable duties program to be provided to me for approval

I have discussed injury requirements and return to work with the patient and ☐ Employer ☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name Peter Hawes Email admin@ourmedicalashmore.com.au
Practice/hospital Our Medical Ashmore Phone 0756475000 Date 01/07/2024
Postal Address Signature
566 Olsen Avenue, Ashmore
QLD 4214

Further information www.worksafe.qld.gov.au/medicalsupport

All enquiries (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation insurer is authorised to collect the information on this form to process the claimant's application for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or any other workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other medical, allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2016, pursuant to section

588 of the Act. © State of Queensland (Queensland Treasury) 2016



Work capacity certificate - workers' compensation

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION Work is an important part of recovery. In most cases, early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. Worker receiving continued support is three times more likely to regain their capacity to work.

Part A - Patient details

Name Cory Shepherd

Mobile number 0422438627

Occupation (if known) Admin worker

Claim number S23LW14203

Date of Birth 11/01/1991

☒ New claim

Patient's employer Qld Health

☐ Claim is report only

Part B - Injury details

Date of examination 01/07/2024

Patient's stated date of injury 18/06/2024

Patient was first seen at this practice/hospital for this injury/disease on 01/07/2024

The patient is/was suffering from (List all work-related diagnoses, if symptoms only, tick "Provisional diagnosis") ☐ Provisional diagnosis

Patient's stated mechanism of injury harassment at work

anxiety, stress

Is this consistent with your clinical findings?
☒ Yes ☐ Unclear

Describe mechanism in detail

Pre-existing factors or condition

aggravated (if not previously supplied)

ongoing breaking of workplace rules by bosses, victimizing him

Part C - Treatment plan

Patient requires/d treatment from 01/07/2024 to 09/08/2024

to be reviewed again on 09/08/2024

☐ No further review

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Specialist/GP

Name/discipline

Details (specify)

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F.

☐ If suitable duties available, can return to some form of work from

☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work
If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

Estimated time to return to some form of work duties unknown

Estimated time to return to full duties unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)

Is functional ability affected by injury/condition?

No Yes Note any restrictions (if relevant)

What patient can do (if "yes" is selected)

Lower limb

No

Upper limb

No

Hand function

No

Spinal function

No

Cognition/psychosocial functioning

No

Driving a car

No

Operating machinery/heavy vehicle

No

Manual tasks

No

Other

No

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

☐ I require a suitable duties program to be provided to me for approval
I have discussed injury requirements and return to work with the patient and

☐ Employer

☐ Insurer

☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

Name Ki Pang

Practice/hospital Our Medical Ashmore

Postal Address

588 Olsen Avenue, Ashmore, QLD 4214

Further information www.worksafe.qld.gov.au/medicalsupport

Further information (medical/dental/nurse practitioner, patient, employer) 131 362 123

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation insurer is required to collect the information on this form to

claimant's application for compensation. The information contained in this form may be disclosed to the claimant's insurer, another insurer, medical or allied health professional, or any other workers' compensation authority in the jurisdiction. The claimant may be contacted by the insurer about the injury. This form was approved by the Workers' Compensation Regulation Authority on 31 May 2016.

666 of the Act. © State of Queensland (Queensland Treasury) 2016

Work capacity certificate – workers' compensation

Form 132M – Version 1

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity to work.

Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name: Cory Shepherd Date of Birth: 11/01/1991
Mobile number: 0422438627 Claim number: S23LW142013 ☐ New claim ☐ Claim is report only
Occupation (if known): Admin worker Patient's employer: Qld Health

Part B - Injury details

Date of examination: 11/08/2024 Patient's stated date of injury: 18/06/2024 Patient was first seen at this practice/hospital for this injury/disease on: 01/07/2024

The patient is/was suffering from (List all work-related diagnoses): anxiety, stress

If symptoms only, tick "Provisional diagnosis" ☐ Provisional diagnosis

Patient's stated mechanism of injury: harrasment at work

Is this consistent with your clinical findings?

☒ Yes ☐ Unclear

Describe mechanism in detail: ongoing breaking of workplace rules by bosses, victimizing him
Pre-existing factors or condition: aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from 11/08/2024 to 08/09/2024 to be reviewed again on 08/09/2024 ☐ No further review

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Name/discipline: Details (specify):
Specialist/GP

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F. ☐ If suitable duties available, can return to some form of work from ☒ No functional capacity for any type of work until 8/9/24

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

stressed and anxious,

Estimated time to return to some form of work duties: unknown

Estimated time to return to full duties: unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	Note any restrictions (if relevant)	What patient can do (if "yes" is selected)
Lower limb	No		
Upper limb	No		
Hand function	No		
Spinal function	No		
Cognition/psychosocial functioning	No		
Driving a car	No		
Operating machinery/heavy vehicle	No		
Manual tasks	No		
Other	No		

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

I require a suitable duties program to be provided to me for approval
have discussed injury requirements and return to work with the patient and ☐ Employer ☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name: Peter Hawes
Practice/hospital: Our Medical Ashmore
Postal Address: 16 Olsen Avenue, Ashmore
Postcode: 4214

Email: admin@ourmedicalashmore.com.au
Phone: 0756475000 Date: 11/08/2024
Signature: [Signature]

For further information www.worksafe.qld.gov.au/medicalsupport
enquiries (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation insurer is authorised to collect the information on this form to process the claimant's application for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or any other workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other medical, allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2018, pursuant to section 585 of the Act. © State of Queensland (Queensland Treasury) 2016



Work capacity certificate – workers' compensation

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and well-being. Practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity. Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name: Cory Shepherd
 Mobile number: 0422438627
 Occupation (if known): Admin worker
 Claim number: S33LW142013
 Date of Birth: 11/01/1991
☐ New claim
☐ Claim is report

Part B - Injury details

Date of examination: 11/08/2024
 Patient's stated date of injury: 18/06/2024
 Patient was first seen at this practice/hospital for this injury/disease on: 01/07/2024
 The patient is/was suffering from (List all work-related diagnoses. If symptoms only, tick "Provisional diagnosis") ☐ Provisional diagnosis: anxiety, stress
 Patient's stated mechanism of injury: harassment at work

Describe mechanism in detail: ongoing breaking of workplace rules by bosses, victimizing him
 Pre-existing factors or condition: aggravated (if not previously supplied)
 Is this consistent with your clinical findings? ☒ Yes ☐ Unclear

Part C - Treatment plan

Patient requires/d treatment from: 08/09/2024 to 08/10/2024 to be reviewed again on: 08/10/2024
 Treatment needs time away from this environment
 I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes
 Referrals: ☒ Diagnostic ☐ Allied Health ☐ Name/discipline: Psychiatrist
 Specialist/GP: Details (specify)

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F.
☐ If suitable duties available, can return to some form of work from
☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work
 If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)
 stressed and anxious
 Estimated time to return to some form of work duties: unknown
 Estimated time to return to full duties: unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) No change since last certificate
 Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.
 Function/task (patient's usual functional ability):
 Lower limb: No
 Upper limb: No
 Hand function: No
 Spinal function: No
 Cognition/psychosocial functioning: No
 Driving a car: No
 Operating machinery/heavy vehicle: No
 Manual tasks: No
 Other: No
 Is functional ability affected by injury/condition? ☒ Yes ☐ No
 Note any restrictions (if relevant):
 What patient can do (if "yes" is selected):

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)
 What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)
 Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)
 I require a suitable duties program to be provided to me for approval
 We discussed injury requirements and return to work with the patient and ☐ Employer ☐ Insurer ☐ Rehabilitation
 G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.
 Peter Hawes
 Email: admin@ourmedicalashmore.com.au
 Phone: 0756475000 Date: 08/09/2024
 Signature: [Signature]

Information: www.worksafe.qld.gov.au/medicalsupport
 (medical/dental/nurse practitioner, patient, employer) 1300 362 128